

HealthConnect Clinic Experience Lab

Translating Operational Findings into Actionable Management Strategies

Operational Foundation: Based on audited analytics (\$N = 5,000\$ records, 48.46% no-show rate, 29.64-day mean lead time, 10.11 km average travel distance, and 24.19-minute wait times).

Actionable Operational Recommendations

1. Combat Lead-Time Attrition via Staggered Multi-Channel Cadences

Finding: Appointments booked weeks or months in advance suffer from severe memory decay and schedule abandonment.

- Implement automated, escalating SMS-first reminders for appointments booked > 14 days in advance.
- Deploy structured check-in sequences at 7 days, 3 days, and 24 hours prior requiring simple confirmation replies (e.g., "Reply C").

2. Deploy Dynamic, Risk-Based Overbooking Models

Finding: A 48.46% no-show baseline results in nearly half of reserved clinic capacity sitting idle daily.

- Safely introduce a controlled overbooking matrix specifically targeting high-risk, long-lead time booking slots.
- Cap overbooking ratios dynamically based on historical failure rates to maximize throughput without causing excessive staff burnout.

3. Mitigate Geographic and Waiting-Time Friction

Finding: Long travel distances (10.11 km avg) compound heavily with extended on-site queues (24.19 mins avg).

- Establish a Priority Fast-Track Check-in for patients traveling > 10 km to offset travel fatigue.
- Smooth out peak arrival blocks to protect patients from long waiting-room delays that incentivize future drop-offs.

4. Establish a Behavioral Flagging System for Repeat Offenders

Finding: Patients with a prior history of unnotified absences exhibit a drastically heightened probability of subsequent defaults.

- Tag accounts with prior unnotified absences as "High-Risk / Repeat No-Show" within the management system.
- Route flagged profiles for direct administrative phone verification 48 hours prior or mandate shorter scheduling windows.